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TCGA-XF-A9SW-01A-PR

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RADICAL CYSTOPROSTATECTOMY WITH ILEOCONDUIT AND BILATERAL PELVIC LYMPH NODE
DISSECTION:

RIGHT DISTAL URETER (A):

FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

*ICDO.3
Carcinoma, urothelial NOS
8120/3
Site Bladder NOS
C67.9
7/13/14*

LEFT DISTAL URETER (B):

FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

BLADDER/PROSTATE (C):

BLADDER:

- INVASIVE UROTHELIAL CARCINOMA (4.5 X 2.3 X 1.5 CM), POORLY DIFFERENTIATED, GRADES 3 & 4/4, INVOLVING FULL THICKNESS OF BLADDER WALL WITH EXTENSION INTO PERIVESICAL SOFT TISSUE
- UROTHELIAL CARCINOMA IN SITU ADJACENT TO INVASIVE UROTHELIAL CARCINOMA
- LYMPHOVASCULAR INVASION IDENTIFIED
- DIVERTICULA (2), ONE WITH ASSOCIATED BLADDER STONE AND EARLY NONINVASIVE PAPILLARY UROTHELIAL CARCINOMA, GRADE 2-3/4
- TRIGONE / BLADDER NECK, ACUTE AND CHRONIC INFLAMMATION WITH ASSOCIATED UROTHELIAL ATYPIA, FAVOR REACTIVE
- RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY

PROSTATE:

- BILATERAL, MODERATE TO POORLY DIFFERENTIATED PROSTATIC ADENOCARCINOMA, GLEASON'S SCORE 7 (4+3), EXTENDING INTO BUT NOT THROUGH CAPSULE AND NOT INVOLVING PERIPROSTATIC SOFT TISSUE [SEE COMMENT]
- PROSTATIC URETHRA, NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED
- RESECTION MARGINS, FREE OF MALIGNANCY AND UROTHELIAL DYSPLASIA
- BILATERAL SEMINAL VESICLES, PERIVESICLE FIBROSIS WITHOUT MALIGNANT INVOLVEMENT

LEFT PELVIC LYMPH NODES (D):

- METASTATIC UROTHELIAL CARCINOMA IDENTIFIED IN ONE OF 11 LYMPH NODES EXAMINED (1/11)

RIGHT PELVIC LYMPH NODES (E):

- METASTATIC UROTHELIAL CARCINOMA IDENTIFIED IN THREE OF EIGHT LYMPH NODES EXAMINED (3/8) [LARGEST POSITIVE NODE MEASURING 1.2 CM IN GREATEST DIMENSION]

LEFT PROXIMAL URETER (G):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

RIGHT PROXIMAL URETER (H):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LYMPH NODE SUMMARY: METASTATIC CARCINOMA IDENTIFIED IN FOUR OF 19 LYMPH

SURGICAL PATHOLOGY REPORT

NOSES EXAMINED

PATHOLOGIC TNM STAGE: **BLADDER:** pTis,Ta,T3bN2MX
 PROSTATE: pT3aN0MX

COMMENT:

Representative sections of invasive urothelial carcinoma are submitted for p53 assay by immunohistology, results of which will be issued in an addendum.

There are changes in the prostatic adenocarcinoma that suggest prior therapy (radiation, hormone or both). And while the bulk of the carcinoma exhibits either fused or separate but distinct malignant glands, there is a small component of tumor characterized by individual infiltrating malignant cells. Finally, tumor is focally seen in and around skeletal muscle fibers in the proximal anterior prostate, with no associated benign glandular tissue seen in the same area. Although the capsule in this area is poorly defined or virtually nonexistent, the changes are interpreted as representing prostatic adenocarcinoma outside the prostate.

SPECIMEN SOURCE:

- A: "Rt. distal ureter F/S"
- B: "Lt. distal ureter F/S"
- C: "Bladder/Prostate"
- D: "Lt. pelvic lymph nodes"
- E: "Right pelvic lymph nodes"
- F: "Left ureter proximal"
- G: "Rt. proximal ureter"

CLINICAL INFORMATION:

Pre-op Dx: TCC of Bladder

Post-op Dx: Same

GROSS EXAMINATION:

A: The specimen is received fresh from the O.R. and labeled "Rt. distal ureter F/S". It consists of a segment of ureter measuring 0.3 cm in length x 0.4 cm in diameter. The specimen is entirely submitted for frozen section in AFS.

B: The specimen is received fresh from the O.R. and labeled "Lt. distal ureter F/S". It consists of a segment of ureter measuring 0.4 cm in length x 0.4 cm in diameter. The specimen is entirely submitted for frozen section in BFS.

C: The specimen is received fresh from the O.R. and labeled "Bladder/Prostate". It consists of a radical cystoprostatectomy specimen measuring overall 20.5 x 14 x 3.2 cm. The peritoneum measures 18 x 14 x less than 0.1 cm. It is tan-pink smooth, glistening and grossly unremarkable. The bladder measures 6.5 x 6 x 5 cm. The surgical margins are inked black. Opening the bladder anteriorly reveals an ulcerated mass on the posterior wall, 1.5 cm above the trigone measuring 3.5 x 2.3 cm in area. On sectioning, it is solid and involves the full thickness of the bladder wall and grossly extends into the perivesical fat. It is 1.5 cm in depth and 0.8 cm from the peritoneal surface. No ureter involvement is identified. The involved bladder wall measures 1 cm in thickness and the uninvolved bladder wall measures 1.5 cm in thickness. The uninvolved bladder wall's appearance is hypertrophic. There are two diverticula identified, one in the right anterior wall (opening 2 cm above right ureterovesical junction) and the other in the dome. The

SURGICAL PATHOLOGY REPORT

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right anterior diverticulum contains a bladder stone measuring 1.2 cm in diameter. It is star-shaped with multiple spikes. The remaining bladder mucosa is unremarkable. The attached right ureter measures 4.5 cm in length x 0.6 cm in circumference and the attached left ureter measures 3.5 cm in length x 0.4 cm in circumference. Both are grossly normal.

The prostate measures 3.5 x 3 x 3 cm. The prostate is opened anteriorly to reveal the prostatic urethra to measure 3 cm in length x 2 cm in circumference. The verumontanum measures 0.8 x 0.6 x 0.5 cm. The prostatic urethra mucosa is tan-pink, smooth and glistening without excavation. Serial sections of the prostate are grossly unremarkable. The right seminal vesicle measures 2 x 1.5 x 1 cm and the left seminal vesicle measures 2 x 1.5 x 1 cm. They are ill defined being enveloped in fibrous tissue. Representative sections are submitted in 27 cassettes.

D: The specimen is received in formalin and labeled "Lt. pelvic lymph nodes". It consists of multiple segments of yellow fatty tissue measuring in aggregate 6 x 4.5 x 1.7 cm. Multiple lymph nodes are identified ranging in size from 0.5 cm to 2.3 cm in greatest dimension. The specimen is entirely submitted in four cassettes.

E: The specimen is received in formalin and labeled "Right pelvic lymph nodes". It consists of multiple segments of yellow fatty tissue measuring in aggregate 4.5 x 4 x 1.5 cm. Multiple lymph nodes are identified ranging in size from 0.2 cm to 1.8 cm in greatest dimension. The specimen is entirely submitted in four cassettes.

F: The specimen is received in formalin and labeled "Left ureter proximal". It consists of a segment of ureter with attached soft tissue measuring 2.2 cm in length x 1.1 x 0.6 cm in cross. The specimen is serially sectioned and entirely submitted in one cassette.

G: The specimen is received in formalin and labeled "Rt. proximal ureter". It consists of a segment of ureter with attached adipose tissue measuring 2.9 cm in length x 0.8 x 0.4 cm in cross. The specimen is serially sectioned and entirely submitted in one cassette.

SECTIONS:

AFS: frozen section, right distal ureter
 BFS: frozen section, left distal ureter
 C1: urethral margin
 C2: anterior wall diverticulum
 C3: anterior wall
 C4: right ureterovesical junction
 C5: right lateral wall
 C6-7: dome and diverticulum
 C8-11: posterior wall / ulcerated mass
 C12: left lateral wall
 C13: left ureterovesical junction
 C14: trigone/bladder neck
 C15: distal prostate
 C16-17: second distal prostate
 C18-19: mid prostate, right and left
 C20-21: second mid prostate, right and left
 C22-23: proximal prostate, right and left
 C24-25: posterior of third proximal prostate
 C26: possible right perivesical lymph nodes
 C27: possible left perivesical lymph nodes
 D1-2: left pelvic lymph nodes
 D3-4: remaining tissue
 E1: right pelvic lymph nodes
 E2: one lymph node, bisected
 E3-4: remaining tissue

SURGICAL PATHOLOGY REPORT

F: left ureter proximal
G: right proximal ureter

INTRAOPERATIVE FROZEN CONSULTATION:

AFS: Right distal ureter
- no high grade atypia or tumor identified

BFS: Left distal ureter
- no high grade atypia or tumor identified

MICROSCOPIC EXAMINATION:

A-B: See final microscopic-diagnosis.

C: Sections of the bladder show a poorly differentiated invasive urothelial carcinoma, grades 3 & 4/4 (C8-11), with a minor component exhibiting glandular features. The tumor involves the full thickness of the bladder wall and penetrates into the perivesical soft tissue (C9) with evidence of lymphovascular space invasion (C11). All surgical margins are free of tumor. Focal urothelial carcinoma in situ is present adjacent to invasive urothelial carcinoma (C10). The ureters and bladder neck are free of tumor. No evidence of prostatic involvement by urothelial carcinoma is seen.

Sections of the prostate show bilateral, moderate to poorly differentiated adenocarcinoma, Gleason's score 7 (4+3), with a minor (less than 5%) component of Gleason's grade 5 tumor. The tumor is confined largely to the prostate but in one site anteriorly in the left proximal prostate the tumor extends into extraprostatic skeletal muscle (C23). No seminal vesicle involvement is identified. All surgical margins are free of tumor.

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted	☒	
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 1/3/2014	